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Project #56: HFH Acute Care PT and OT Throughput Initiative

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Introduction

Problem #1:

- Rehab lacks qualitative metrics for Key Performance Outcomes

Problem #2:

- Improving Rehab throughput/appropriate utilization would assist in decreasing LOS for the patient.

Issue
1 Delayed order placement
2 Inappropriate orders (totally independent or dependent at baseline)
3 Delayed notes for discharge pending
4 Delayed TAT for non-pathway patients (med/surg)
5 Reduced Frequency of visits for non-pathway (med/surg)
5 Anchor Bias to DC recommendation - PT and OT
7 Anchor Bias to DC recommendation - Provider or CM
3 IPR recommendations: High rate of denial
3 Patients with IPR have High LOS due to length of time for authorization (also due to severity of illness typically)
3 Patients going to SNF or IPR typically have higher LOS
1 Lack of Analytics Data
2 Barriers to Productivity - Lack of available Equipment (chairs, walkers)
3 Barriers to Productivity - Co Treats
4 Barriers to Productivity -Inappropriate consults and discharge pending

Aims

- Improve TAT and Frequency to in turn improve Rate of DC to post-acute facility in turn to reduce LOS
 - Reduce demand for services deemed unnecessary / See more of “right” patients
 - Reduce demand for follow ups per therapist prescription (educate and train at time of evaluation)
 - Reduce Anchor Bias for Providers and Mid-Levels due to verbiage and highlighting of Anticipated DC disposition.
 - Reduce delays in LOS due to IPR Recommendations

Initiatives started in 2022

- Why Not Home culture change: “What will it take for patient to go home”
- Created Patient List Headers to inform CM, Providers, anyone of mobility scores, discharge recs, therapist communication
- Targeted training to: Reduce cotreats; Alternating care between PT and OT; Reduce Home with 24 hour recommendation
- Education to Providers/CM that DC Rec is a recommendation only based on what is clinically happening with the patient; PPT, Pocket Card, Postings in documentation rooms on floor with QR code for Flow chart.
- Morning review: GPU - assign those admit over 24 hours first ; Triage by ML and chart review // Morning review ICU - chart review ; round with fellows; collab with nursing (early rehab protocol to screen all admits to intervene within 24 hours of medical stability
- ED/Obs: do not assign unless rounds with CM or provider/CM calls
- All calls / escalation go thru screening questions prior to assignment by rehab assistant or charge therapist
- Annual Provider and mid level education - when not to consult, Reduce requests fo PT and OT to change recs from Home to SAR when patient can't go home ; The patient is given information to make an informed decision, but ultimately the choice is theirs; Advocating for Nurse Mobility
- Added 5 point CM reference to reduce delays in IPR auth if probability of acceptance is low

Methods

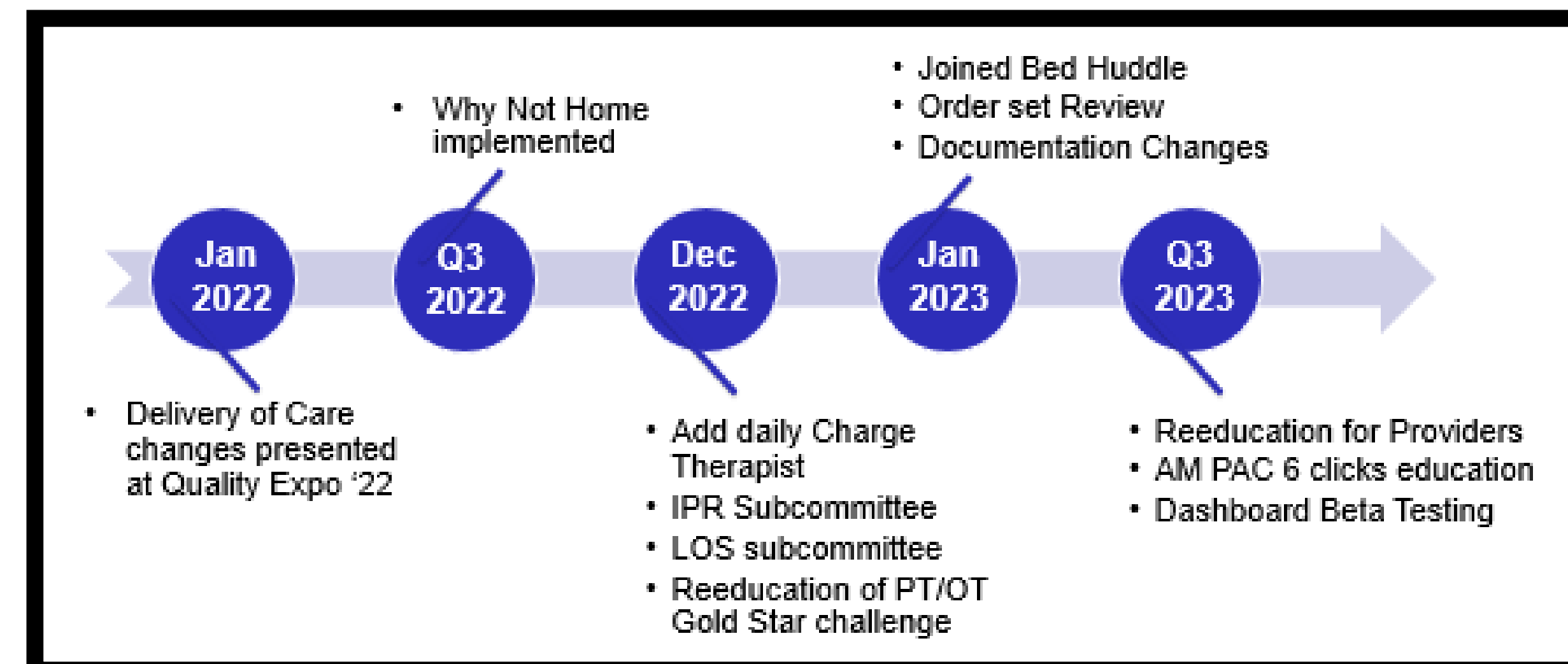


Figure 1. Timeline

January 2023

- Reduce Anchor Bias: Eliminate highlighting DC Recommendation in Documentation; Add DME and assistance needed at DC for all patients even if it is anticipated the patient will need SAR placement
 - Reviewed ordered sets – prechecked on 22 ortho/neuro/cardiac/early rehab only
- August 2023:** Clinical Dashboard launches

Mid-December 2022

- Added daily Charge Therapist – triaging requests, screens evals, educate case by case or appropriateness
- Gold Star Challenge / Reeducation
- Consider whether the discharge rec is too conservative
 - Remind PT/OT staff to consider resources that may be available to the patient in their home that could make the home discharge safe.
 - Utilize Case Manager as a resource (SNF at home, Home PT/OT, is patient eligible for a care worker, utilizing WNH)
 - It is important to acknowledge that these options are most successful when all parties participate (i.e. if the recommendation implies IPR, that should be attempted barring any insurance limitations)
 - Reeducated PT OT on criteria for IPR, use of PAR rec; Inservice with PMR/IPR

Results

- Measures : Compare Jan-Dec 2022 to Jan to Dec 2023: LOS/ % of pts to SNF; TAT within 48hours; Freq;
- DC recommendations to IPR and SAR have reduced
- Despite increase in volume, % of census with PT/OT decreased
- TAT improved ~1-2 hours
- # Visits per patient decreased while Frequency overall increase
- Hospital LOS for patients with PT reduced by 1.11 days and 0.97 days for OT. This outpaced reduction for overall Hospital LOS reduction (-0.77 days)**

Anticipated DC Disposition	% PT/OT Patients		
	2021	2022	2023
Home (any)	59.90%	62.53%	65.57%
IPR	14.27%	11.12%	9.58%
Post Acute Rehab	4.00%	3.16%	2.70%
SAR	21.83%	23.19%	22.15%
Total count	14,349	13,983	13,533

DC Rec Dashboard, Inpatient Class. 1/1/21-12/31/23

Table 1 Anticipated Discharge Disposition (Rec)

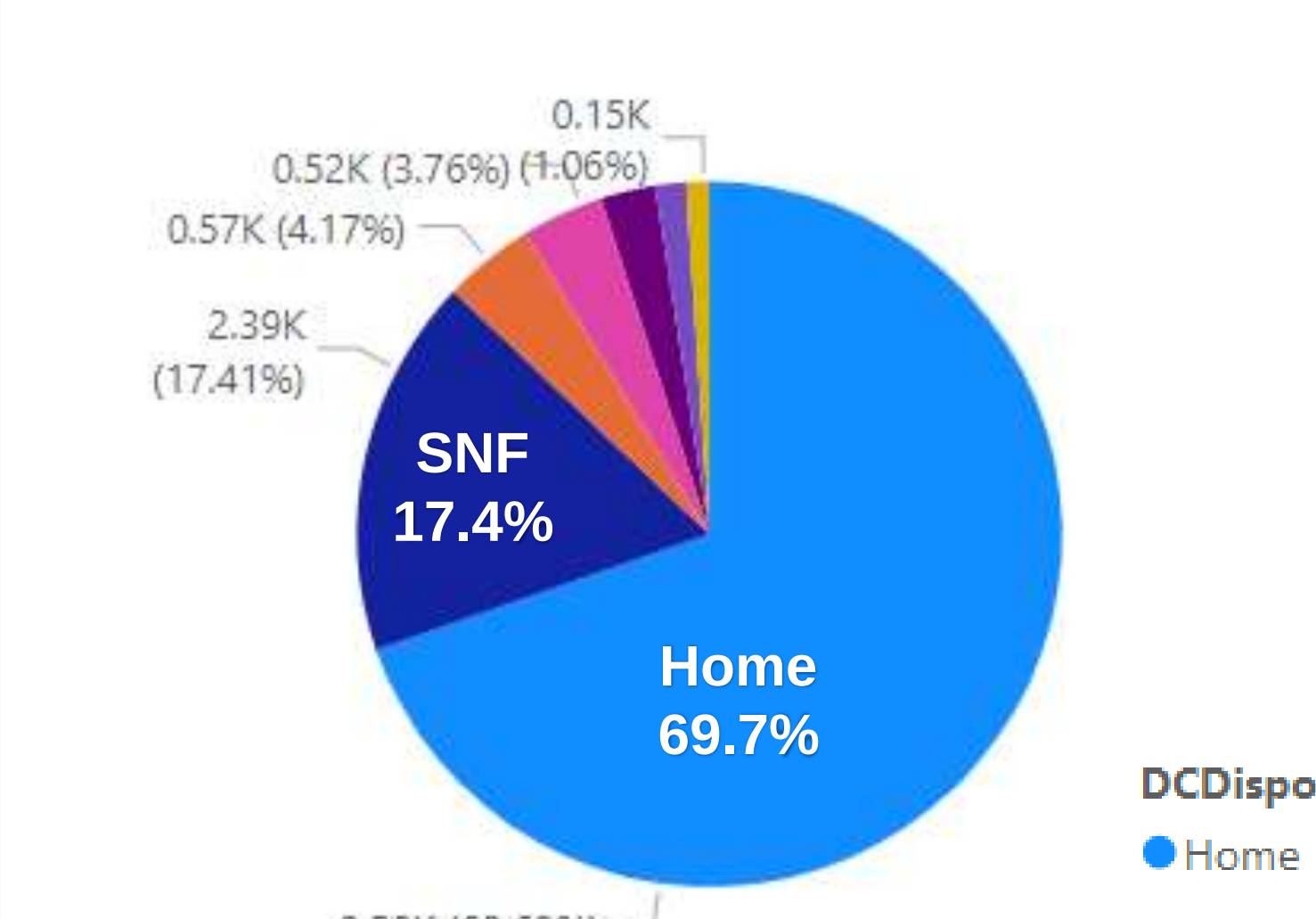
	Variable	2022	2023	Difference
Physical Therapy	Patients Admitted, Inpatient Status, HFH Detroit	27,725	30,175	2,450
	Patients with 1 or more PT Orders , n (% of Pts Admitted)	16,202 (58.4%)	16,940 (56.1%)	738 (-2.3%)
	Patients with 1 or more PT Visits , n (% of Pts Admitted)	14,135 (51.0%)	14,612 (48.4%)	477 (-2.6%)
	Patients Never Seen by PT/diverted, n (% PT orders)	2,067 (12.8%)	2,328 (13.5%)	293 (+0.7%)
	TAT, PT Order to First PT Note, hours	34.0	33.1	-0.85
	TAT, PT Order to First PT Visit, hours	40.1	38.1	-2.02
	Total PT Visits (all patients)	43,054	41,432	-1622
	LOS from First PT Visit to DC (PT LOS), days	7.26	6.31	-0.95
	Frequency, PT Visits Per PT Patient, mean	3.05	2.87	-0.17
	Frequency Ratio (Avg PT Visits per PT LOS)	0.42	0.45	0.03
	Frequency Ratio converted to days out of 7 a pt received PT	2.94	3.12	0.19
	Hosp LOS (All Admitted), days	7.02	6.25	-0.77
	Hosp LOS (Patients with PT visits), days	10.02	8.91	-1.11
	CMI (PT patients)	2.92	2.81	-0.11
Occupational Therapy	Patients with 1 or more OT Orders , n (% of Pts Admitted)	15,975 (57.6%)	16,752 (55.5%)	777 (-1.9%)
	Patients with 1 or more OT Visits , n (% of Pts Admitted)	13,706 (49.4%)	13,628 (45.2%)	78 (-4.2%)
	Patients Never Seen by OT/diverted, n (% OT orders)	2,269 (14.2%)	3,124 (18.7%)	855 (+4.5%)
	TAT, OT Order to First OT Note (hours)	33.9	33.7	-0.20
	TAT, OT Order to First OT Visit (hours)	41.3	40.2	-1.10
	Total OT Visits (all patients)	40,223	37,540	-2683
	LOS from First OT Visit to DC (OT LOS), days	7.33	6.48	-0.85
	Frequency, OT Visits Per OT Patient, mean	2.93	2.75	-0.18
	Frequency Ratio (Avg OT Visits per OT LOS)	0.4	0.42	0.02
	Frequency Ratio converted to days out of 7 a pt received OT	2.8	2.97	0.17
	Hosp LOS, days (All DC)	7.02	6.25	-0.77
	Hosp LOS, days (Patients with OT visits)	10.18	9.21	-0.97
	CMI (OT patients)	2.92	2.86	-0.06

Source: Clinical-Quality HFHS Rehab Dashboard. Applied Filters: Inpatient Status; Patients Admitted during calendar year 1.1.2022-12.31.2022 or .2023-12.31.2023. Patients Discharged by 12.31.2023

LOS=Length of Stay; DC=Discharge; TAT=Turn Around Time

Table 2 PT and OT Outcome Variables

Actual DC Disposition (2022)



Actual DC Disposition (2023)

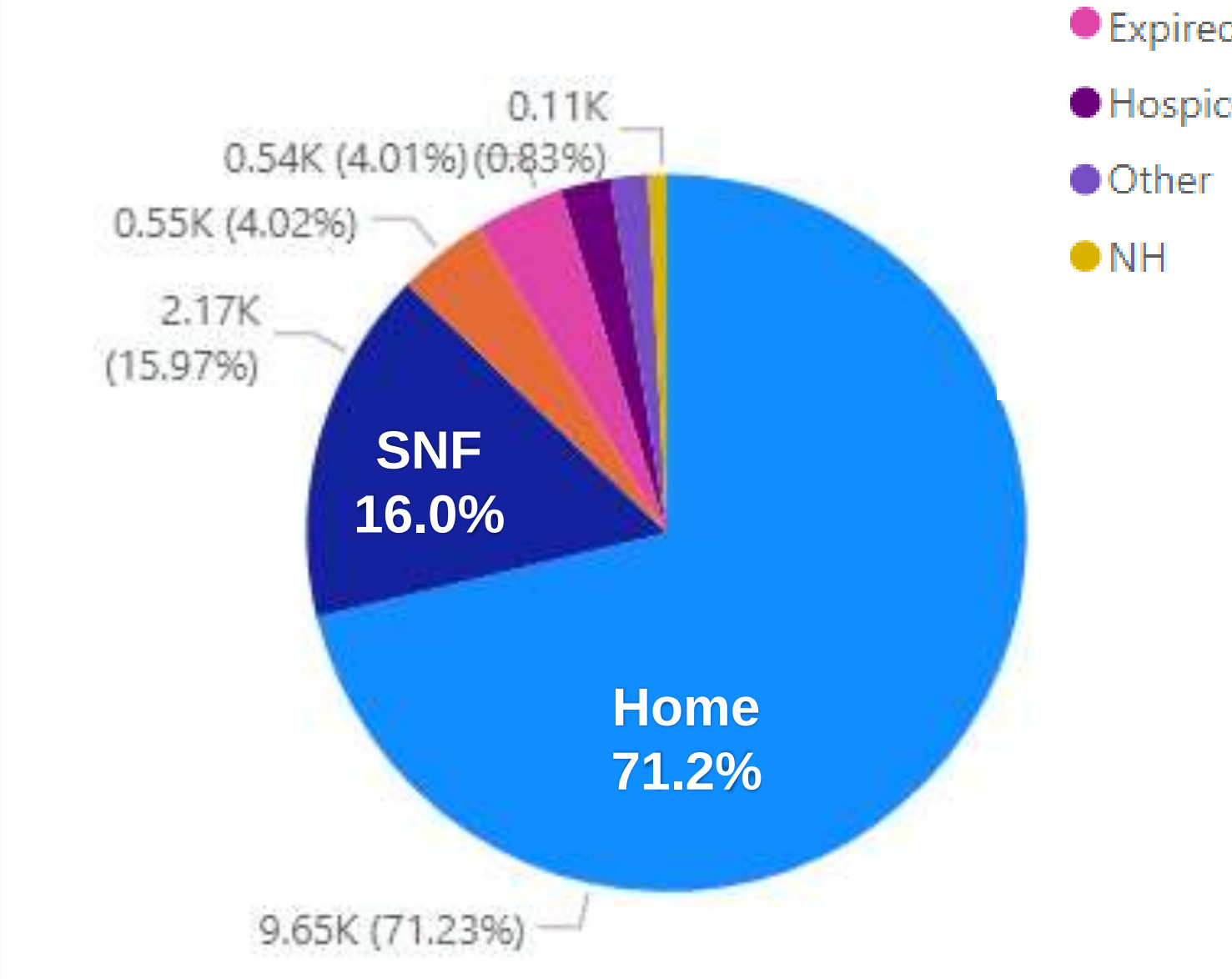


Figure 1. Actual Discharge Disposition

Source: Clinical-Quality Rehab Dashboard

Keys to Success

- Participation in LOS Subcommittees including the Throughput Subcommittee and DC Planning Subcommittee
- Multidisciplinary Team Projects including Provider, Case Manager, Nurse Managers, front line PTs and OTs
- Robust EMR Data Collection designed 2014 to present
- Collaboration with System Analytics
- Collaboration with System IPR and HFH Case Management

Spread

- System Acute Care EPIC Advisory Team input received and shared
- Clinical Analytic Dashboard includes all 5 acute care hospitals

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